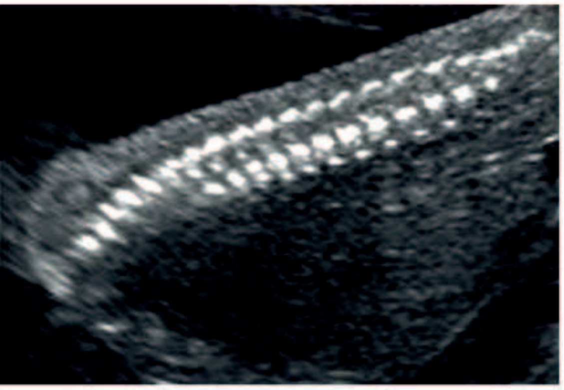
Taking a look The sonographer will need to change the position of the handheld sonograph as the ultrasound takes place—getting a good look at your baby from all angles.

Looking at your baby's ultrasound At the 20-week ultrasound the sonographer will look closely at the structure of your baby's organs, and will also study external features, such as your baby's facial features.



Your baby's vertebrae and spinal cord All the bones along the length of your baby's spine will be checked to see that they are properly aligned and that the spinal cord is fully enclosed in skin to eliminate the possibility of a neural tube defect such as spina bifida.

POSSIBLE ANOMALIES

The table below details specific checks your baby will be given. It's important to remember that the anomalies are all very rare. The chance that problems are detected—the pick-up rate—is affected by factors such as obesity, scar tissue from a previous C-section or surgery, and the baby's position.

CONDITION	DESCRIPTION	PROGNOSIS	PICK-UP RATE
Neural tube defects (occur in six out of 10,000 births)	Anencephaly and spina bifida are neural tube conditions in which the skull, spine, and/or brain don't develop fully.	There is no treatment for anencephaly and the baby will die before or shortly after birth. Spina bifida can be a limiting condition, but with considerable support, children can go on to lead active lives.	98 percent for anencephaly; 90 percent for spina bifida
Hole in the abdominal wall (occurs in 4–5 out of 10,000 births)	This can result in gastroschisis or exomphalos where part of the intestine and possibly the liver develops outside the baby's body.	Both conditions are potentially treatable with surgery as soon as the baby is born. Some babies with exomphalos also have heart defects.	98 percent for gastroschisis; 80 percent for exomphalos
Diaphragmatic hernia (occurs in 4 out of 10,000 births)	A hole in the baby's diaphragm means that his lungs aren't able to develop properly.	Around 50 percent of babies with this will die as soon as they are born, because their lungs are simply too underdeveloped even for emergency surgery at birth.	60 percent
Cleft lip and cleft palate (occurs in 1 out of 1,000 births)	Your baby's lips grow in two parts, sometimes including the palate inside the mouth. If the parts don't join together properly, or at all, the baby will develop a cleft lip and palate.	Neither cleft lip nor palate are life-threatening. Your baby will have surgery usually within six months after birth, to knit the two parts together, often with little scarring.	75 percent
Major heart defects (occur in 3–4 out of 1,000 births)	Enlarged heart chambers, valves that allow a two-way flow of blood and holes in the heart are some of the congenital heart problems that are classified as major defects.	Only when a specialist has diagnosed the nature of the baby's heart problem can any real indication be given of his likely prognosis.	50 percent
Lack of kidneys (occurs in 1 in 10,000 births)	Medically known as bilateral renal agenesis, this means that the baby has developed without any kidneys, and possibly a bladder.	Sadly, we need our kidneys to survive so babies with this condition die as soon as or before they are born.	84 percent
Lethal skeletal dysplasia (occurs in 1 in 10,000 births)	With this, the baby's bones don't develop fully, making the torso of the body very short, as well as giving the baby short limbs.	The short torso means that the baby's lungs don't develop fully, making it very hard for a baby to survive after birth.	90 percent